

So you're thinking about physical therapy

The short version — written for you, not about you.

Everything that matters, none of the bits written for your parents.

We scored twenty-seven careers on how exposed they are to AI.

Hands-on physical therapy: **2.5 out of 10**. That's the lowest score of anything we measured except surgery.

Entry-level software development: 8.1.

So the AI question here is basically settled. **The question that actually matters is what it costs to qualify** — and that varies more than you'd believe.

The 60-second version

- Physical therapy is among the most protected work in the entire index. That part isn't close.
 - Occupational therapy 2.6, speech therapy 3.1, respiratory therapy 3.1 — all excellent.
 - **The real risk is the debt**, and it depends almost entirely on which school you attend.
 - The same degree costs over \$300,000 more at some programs than others.
 - And there's an alternative route most people never hear about.
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Why it's so protected

You can't do it through a screen. Hands on a body. Feeling resistance, guarding, compensation. Adjusting in real time to how someone actually moves.

We rate this **9.5 out of 10** on physical requirement — the highest rating we give any career in the whole index.

It's licensed, with mandatory supervised clinical hours that can't be automated or offshored.

It's a relationship over months, not a transaction. Most of rehab is getting someone to actually do the work when it hurts.

And every body responds differently. The judgment about when to push and when to stop is made in the moment, with a person in front of you.

One honest caveat

Of our four protections, physical is the one we rank least durable over a long horizon — and this is the career where that matters most.

You'd practise for about forty years. Licensure and trust move at the speed of law and society; they'll hold. Physical protection moves at the speed of robotics, which is the one thing actively aimed at it.

And there's early evidence worth knowing. Robotic-assisted rehabilitation is showing better motor outcomes than conventional therapy in some protocols. Not equivalent — better.

Two things keep our score where it is: a licensed clinician still prescribes, supervises and adapts in every deployment we found, and the research says routine use is still limited.

The thing to watch is whether robots move from assisting a therapist to replacing therapist hours in structured, protocol-driven rehab. Structured settings are where automation always shows up first. A stroke patient at home, or an athlete with an unusual injury, is last.

The scores

ROLE	SCORE
Physical therapy (hands-on)	2.5
Occupational therapy	2.6
Speech-language pathology	3.1
Respiratory therapy	3.1
Diagnostic imaging technologist	4.4
Records / screen-based allied roles	6.1

Same pattern as everywhere in our index: move the same qualification behind a screen and the exposure roughly doubles.

Now the part that actually matters

The debt.

- Typical DPT graduate: **\$140,000–\$170,000** in total student debt
- Starting salary, outpatient: **\$75,000–\$95,000**
- So a debt-to-first-year-salary ratio of about **1.7 to 2.0**

For comparison: new physicians are at 0.6–0.9x (way more debt, way more salary). Nurse practitioners 0.9–1.3x. Pharmacists 1.5–1.8x.

The APTA — the profession's own body — asked physical therapists for the word they most associate with student debt. The most common answers were "burden," "unfair," "stressful" and "crushing." Around 28% work a second job on top of their PT role.

The number that should choose your school

Someone analysed all **206 DPT programs across 151 institutions.**

- Best debt-to-income ratio: **0.82:1**
- Worst: **4.42:1**
- Median: **1.8:1**

That's a difference of more than \$305,000 in total cost. Same degree. Same licence. Same career.

And it gets worse: federal loans cap out at \$20,500 a year, while the average program has a funding gap of about **\$33,647 a year** above that. Over three years that's roughly \$95,000 in **private** loans — which don't qualify for income-driven repayment or Public Service Loan Forgiveness.

So: go to the cheapest accredited program you get into. The difference in program quality is small. The difference in cost lasts decades.

That's probably the single most valuable sentence in this document.

The route nobody tells you about

Physical therapist assistant.

- **Two-year associate degree** instead of a bachelor's plus a three-year doctorate
 - Median pay around **\$65,510** versus roughly \$80,000–\$90,000 for a PT
 - Average debt around **\$24,000** where it exists at all
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Here's the arithmetic: a PTA working during the years you'd spend in DPT school could have earned close to \$290,000 by the time you qualify — while you'd start with \$140,000+ of debt.

Over a full career the DPT does win. The income gap compounds and the ceiling is higher. But it takes many years to close, and how many depends entirely on which school you picked.

And the AI protection is basically the same — it's hands-on, physical, licensed work.

If what pulls you is the actual hands-on rehab rather than diagnosis and autonomy, this route deserves a real look before you commit to the doctorate.

What we won't soften

Around 70% of physical therapists report moderate to high burnout. But look at what causes it: unrealistic productivity targets, heavy documentation, admin. Not the clinical work.

Which is interesting, because that's exactly the layer AI is removing. This might be the career in our whole index where automation most directly fixes the profession's actual complaint.

It's also physically demanding over decades — standing, lifting, transferring people.

And some patients don't get better. Sitting with that is harder than most students expect.

Does this actually sound like you?

It suits people who:

- **Want to see progress they can measure** — the most-cited thing people love about it
- **Are comfortable with physical contact** and with bodies generally
- **Can motivate someone who doesn't want to be motivated** — that's most of the job
- **Would rather move than sit**
- **Are patient with slow improvement** — rehab is measured in months

Harder if you need quick resolution, want high early earnings relative to what training costs, or find it hard when someone doesn't improve.

What actually matters when picking a program

Total cost of attendance. Calculate the debt-to-income ratio yourself. Nothing else on this list comes close.

- **CAPTE accreditation** — non-negotiable
 - **How many clinical settings** do you rotate through? Setting shapes the career more than the qualification does.
 - **Licensure exam first-time pass rate**
 - Do they teach **rehab technology and AI documentation tools**?
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Two things worth doing

Shadow a therapist in two completely different settings — outpatient orthopaedic and something else, like neuro or paediatrics. They're genuinely different jobs and people rarely realise that until they're in one.

Then find someone five years qualified and ask straight out about their loan payments, and whether they'd choose the same program again. It's an awkward question and the answer is worth a lot.

One last thing

Everything above about money is about **which program**, not about **whether**.

The career itself is one of the safest we measured, the demand is demographic and durable, and watching someone walk again, or lift their arm again, or get back to something they thought they'd lost — very few jobs offer that at all.

Just don't pay \$375,000 for it when someone else is paying \$70,000 for exactly the same licence.

If you want to go further — three things worth twenty minutes

- **Physical Therapists — Occupational Outlook Handbook** — government data on growth and pay. Read the **Physical Therapist Assistants** page right after it; that comparison is the most useful twenty minutes on this decision.

- **Is physical therapy worth the debt? 2026 analysis** — the program-by-program cost breakdown. Read this *before* you decide where to apply, not after.
 - **Impact of Student Debt on the Physical Therapy Profession** — the profession's own research into what the debt does to people. Sobering and worth reading.
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There's a longer version behind this — full scores by role, how they were worked out, the robotics question in detail, what to ask a program, and where we might be wrong. Your parents have it.